

1. Administrative & Study Identification

Full Study Title: Evaluation of Oral Tofacitinib in Children Aged 2 to 17 Years Old Suffering From Moderate to Severe Ulcerative Colitis **Short Title/Acronym:** OVATION Study (A3921210) **Protocol Number:** A3921210 **NCT Number:** NCT04624230 **Posting ID:** 317368507894836993 **Sponsor/Company Name:** Pfizer **Investigational Product:** Tofacitinib (Trade Name: Xeljanz | ATC Code: L04AA29) **Phase of Development:** Phase III **Trial Status:** Recruiting **Medical Monitor:** Pfizer Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective **Primary Objective:** To evaluate the efficacy of tofacitinib based on remission in pediatric participants with moderately to severely active Ulcerative Colitis (UC). **Secondary Objectives:** To evaluate the safety, pharmacokinetics (PK), clinical response, and change in disease activity (Mayo score) of tofacitinib in pediatric participants with moderately to severely active UC.

2.2 Background & Rationale To address the therapeutic need for pediatric patients (aged 2 to 17 years) suffering from moderate to severe Ulcerative Colitis. This study aims to evaluate oral tofacitinib in children who have had an inadequate response, prior failure, or intolerance to oral or IV corticosteroids, azathioprine, 6-mercaptopurine, or Tumor Necrosis Factor inhibitors (TNFi).

3. Study Design & Methodology

3.1 Design Framework **Study Type:** Interventional **Control Type:** Single-arm **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration **Target N\$:** 120 participants **Number of Sites:** Global (US, EU, and non-US/EU regions) **Estimated Study Start:** 2020 (based on protocol initiation) **Estimated Primary Completion:** 2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Males and females aged 2 to 17 years old (less than 18 years old) and weighing at least 10 kg. 2. Confirmed diagnosis of moderately to severely active Ulcerative Colitis. Defined via screening colonoscopy by a Mayo score of \$

≥ 6 , with a rectal bleeding score of ≥ 1 and an endoscopic subscore of ≥ 2 , alongside a Pediatric Ulcerative Colitis Activity Index (PUCAI) score ≥ 35 . 3. For US/EU: Prior TNFi failure or intolerance. Outside US/EU: Inadequate response or intolerance to oral/IV corticosteroids, azathioprine, 6-mercaptopurine, or TNFi.

4.2 Exclusion Criteria 1. Hemoglobin level < 9.0 g/dL; absolute white blood cell (WBC) count $< 3000/\text{mm}^3$; absolute neutrophil count $< 1200/\text{mm}^3$; absolute lymphocyte count $< 750/\text{mm}^3$; or thrombocytopenia (platelet count $< 100,000/\text{mm}^3$). 2. Estimated bedside Schwartz Glomerular filtration rate (GFR) $\leq 40 \text{ mL/min/1.73 m}^2$; Total bilirubin, AST, or ALT > 1.5 times the upper limit of normal. 3. Positive stool examinations for enteric pathogens, pathogenic ova or parasites, or C. diff; history of bowel surgery including cholecystectomy within 6 months, or appendectomy within 3 months prior to baseline. 4. Diagnosis of indeterminate colitis, isolated proctitis, microscopic colitis, Crohn's disease, or short bowel syndrome.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Open-label tofacitinib 5 mg BID weight-based adult equivalent with the option for an individual dose increase to 10 mg BID weight-based adult equivalent for a limited time if dose escalation criteria are met, prior to returning to 5 mg BID. **Route of Administration:** Oral **Duration of Treatment:** Up to a maximum duration of 180 weeks (comprising an 8-week or 16-week induction phase, a 44-week maintenance phase, and a 24-month extension phase).

5.2 Concomitant & Prohibited Therapy Permitted: Oral 5-Aminosalicyclic acids (ASA) or sulfasalazine; oral corticosteroids equivalent to prednisone up to 1 mg/kg (maximum of 20 mg/day) or budesonide up to 9 mg/day. **Prohibited:** Moderate to potent CYP3A inducers or inhibitors; chronic and frequent use of ant motility agents for the control of diarrhea (e.g., diphenoxylate hydrochloride with atropine sulfate or loperamide).

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Up to 4 weeks Informed Consent, Medical History, Blood & Stool Labs, Endoscopic Evaluation
Induction	Weeks 8 or 16	First Dose, Clinical Assessment, Efficacy/Safety Monitoring	Maintenance Up to Week 44 Efficacy Assessment, Dose Titration (if applicable), Safety Labs
Extension	24 Months	Long-term PK, Efficacy, and Safety Monitoring	End of Study \$+4\$ to \$+12\$ Weeks Follow-up

visit 4 weeks after the last dose, and a telephone contact 8 weeks later for AE/SAE assessment |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Remission following 44 weeks in the maintenance phase. Remission is defined by a central read Mayo score of 2 points or lower, with no individual subscore exceeding 1 point and a rectal bleeding subscore of 0.

Measurement Method: Central read Mayo score.

7.2 Secondary & Exploratory Endpoints 1. Response by Mayo score. 2. Remission by Mayo score. 3. Change from baseline in Mayo score. 4. Safety and Tolerability parameters of tofacitinib (AEs/SAEs) in pediatric patients. 5. Pharmacokinetics (PK) of tofacitinib in pediatric patients.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring throughout the 180-week study period, including physical exams, blood/stool sample collections, and a specific telephone contact 8 weeks post follow-up. **Serious Adverse Events (SAEs):** Routine monitoring and reporting per regulatory standards (typically 24 hours). **Data Monitoring Committee (DMC):** Internal safety evaluation maintained by Pfizer and Clinical Trial investigators.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy; Safety Set for all participants receiving at least one dose of tofacitinib. **Sample Size Justification:** Target enrollment of $N=120$ designed to adequately evaluate the PK, safety, and remission rates in a specific pediatric subpopulation. **Handling of Missing Data:** Standard methodologies (e.g., Non-Responder Imputation or Multiple Imputation) used for missing central read Mayo scores at week 44.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring for data validation and protocol adherence. **Audit Trail:** Assured through eCRF locking mechanisms and central reads for the primary endoscopic outcomes.

11. Study Identifiers & Governance

Primary ID: A3921210 **Registry Number:** NCT04624230 **Posting ID:** 317368507894836993 **Sponsor/Lead Organization:** Pfizer **Study Title:** Evaluation of Oral Tofacitinib in Children Aged 2 to 17 Years Old Suffering From Moderate to Severe Ulcerative Colitis **Official Regulatory Title:** OPEN-LABEL INDUCTION AND MAINTENANCE STUDY OF ORAL CP-690,550 (TOFACITINIB) IN CHILDREN WITH MODERATELY TO SEVERELY ACTIVE ULCERATIVE COLITIS **EMA Product Information:** [EMA URL](#)

12. Clinical Framework (Ulcerative Colitis Specific)

Primary Condition: Moderately to severely active Ulcerative Colitis (UC) **Disease Classification Codes:** MeSH: D003093 | SNOMED ID: 64766004 | HPO: HP:0100279 **Diagnosis Threshold:** Clinically and endoscopically confirmed moderate to severe UC requiring systemic intervention. **Medical Methodology:** Janus Kinase (JAK) Inhibitor Therapy **Optimization Target:** Clinical Remission (Mayo score ≤ 2)

13. Study Procedures & Methodology

Management Pathway: Standard clinical pathway for Pediatric UC evaluating tofacitinib induction and maintenance. **Education Protocol (HCP):** Site initiation visits and standardized central read Mayo score training. **Education Protocol (Patient):** Informed consent (parents/guardians) and assent for age-appropriate pediatric patients. **Quality Audit Mechanism:** Blinded central review of endoscopies to objectively evaluate the primary endpoint.

14. Observation & Follow-Up Schedule

Total Duration: Up to 180 weeks (excluding 12-week post-treatment follow up) **Onsite Visit Cadence:** Routine visits scheduled throughout the 4-week screening, 8/16-week induction, 44-week maintenance, and 24-month extension. **Remote Monitoring Frequency:** Telephone contact 8 weeks after the final 4-week post-treatment follow-up. **Checklist Review Interval:** Sporadic endoscopic evaluations throughout the distinct trial periods.

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				